Revisional Surgery After OAGB — Comparison of Four Surgical Propositions

Patient: M. Mohsin Chowdhury, 59 y, male · BMI □ 27 · OAGB (July 2017, BP limb 155 cm)

Problem: Chronic marginal ulcer with bleeding · bile reflux · hiatal hernia (Hill IV) · gallstones · persistent H. pylori · iron-deficiency anaemia

Propositions compared: Prof. Wei-Jei Lee (Taiwan) · Dr Usama Iqbal (Aga Khan, Karachi) · Dr Murat Üstün (Istanbul) · Dr Fakhrul Islam (advisory opinion)

Prepared from the email correspondence in MMS-ITS/Revision_Surgery · research-dated 8 November 2026

Important — read first. This document is an *educational comparison of the written opinions* contained in the shared correspondence. It is **not medical advice** and is not a substitute for a formal consultation. The scores are a structured judgement of each *proposal as written* (its reasoning, completeness and alignment with current bariatric-revision literature) — **not** a ranking of the surgeons' ability, and not a recommendation to choose any one centre. Any decision must be made with the treating team after a full in-person assessment. Prepared by an AI assistant; verify all clinical points independently.

1. The clinical situation in brief

A 59-year-old man had a one-anastomosis gastric bypass (OAGB / mini-gastric bypass) in 2017 with the gastrojejunostomy at 155 cm. Nine years later he has the classic triad that drives OAGB revision:

Marginal ulcer	Large, chronic, just distal to the gastrojejunostomy; bled in Sept 2026 (Hb fell to 8.2 g/dL).
Bile reflux	Large volume of bile seen in the pouch — the mechanism most associated with OAGB ulceration and reflux.
Hiatal hernia	Hill grade IV (severe).
Gallstones	Multiple 5–6 mm stones, new since 2025; asymptomatic.
H. pylori	Persistently positive after two eradication courses; eradication not yet confirmed.
Anaemia	Recurrent iron deficiency; cold intolerance; on IV iron.
BMI □ 27	Weight is *not* the problem — protecting nutrition and avoiding further weight loss is a goal.

Where the field broadly agrees: for bile reflux + recurrent marginal ulcer after OAGB, converting to a Roux-en-Y configuration (which diverts bile away from the pouch) is the mainstream durable solution; the gallbladder is usually removed in the same sitting because post-RYGB ERCP is very difficult; and the hiatal hernia should be repaired. The *disagreements* between the four opinions are about the details that matter most: whether to resect the ulcer, whether to cut the vagus nerve, and how long the Roux (alimentary) limb should be.

2. The four propositions, side by side

Element	Prof. Wei-Jei Lee	Dr Usama Iqbal	Dr Murat Üstün	Dr Fakhrul Islam
Core operation	OAGB□RYGB; **keep** the old gastrojejunostomy, buttress it with circumferential sutures	OAGB□RYGB; **resect** the ulcerated anastomosis & create a new one	OAGB□RYGB (or reversal); ulcer area "assessed — may need local revision or resection"	OAGB□RYGB; **refashion a new anastomosis**
Ulcer handling	Left in place; healing expected after acid/bile control + sutures; no biopsy	Removed with the anastomosis — cause excised	Judged intra-op; flags that ulcer "should not be assumed bile-only"	Removed by refashioning; sees ulcer as a consequence of bile reflux
Truncal vagotomy	**Yes** — calls acid reduction "the utmost priority"	Not proposed	Not proposed	**Explicitly against** — ulcer is bile-driven, not acid-driven
Roux / limb length	New anastomosis 40 cm below old GJ (to ~60 cm if common channel long)	Standard RYGB; length not specified	Standard RYGB; length not specified	**AL 75 cm + BP 50–60 cm**, specified
Hiatal hernia	Repair + reduce herniated tissue	Repair (routine at centre)	Repair if confirmed appropriate	At least cruroplasty if pouch migration suspected
Gallbladder	Remove if symptoms/preference	**Remove** (hard to treat later complications)	Discuss — stones alone don't mandate removal	**Must be removed** in the same sitting
H. pylori	"Little role" in ulcer; treat any time	Long-term GI treatment, can be post-op	Confirm eradication; don't stop PPI unsafely	May add predisposition; secondary to bile

Element	Prof. Wei-Jei Lee	Dr Usama Iqbal	Dr Murat Üstün	Dr Fakhrul Islam
One stage vs staged	Single stage, no drain/NG, 1–2 d stay	Single stage (all three together)	Single stage if stable; staged if bleeding/anaemia/poor nutrition	Single stage
Stated experience	OAGB since 2000; ~30 of this exact procedure/20 y; recent complication rate 0	~30 revisions (mixed), 25 HH/yr, "no complications"	Volume not given in writing; team-based	N/A — advisory, refers on (suggests USA)
Pre-op work-up	Minimal; no routine extra tests	**Structured lab panel** + anaesthesia teleclinic	**Most thorough**: nutrition, anaesthesia, drug/hypersomnia review, record reconciliation	Defers detail to operating surgeon
Indicative cost / stay	~US$10k lap (US$15k robotic); ~1 week	~PKR 1.5–2.05m (US$5.4–7.4k); 1–2 d + 1–2 wk in country	£5,350 base + £2,000 for HH+chole £7,350; ~10 d	N/A

The three technical fault lines. (1) *Keep vs resect the ulcer* — Lee keeps it (speed/safety, less dissection); Usama and Fakhrul resect it (removes the diseased tissue, allows biopsy, lower recurrence risk but a fresh anastomosis to heal). (2) *Vagotomy or not* — Lee adds truncal vagotomy to cut acid; Fakhrul argues this is the wrong target because the driver is bile, and most current literature treats RYGB diversion (± resection) as the primary fix with vagotomy reserved for refractory cases. (3) *Limb length* — only Lee and Fakhrul commit to numbers; Fakhrul's longer alimentary limb (75 cm) is more explicitly aimed at keeping bile off the pouch, which is the patient's chief concern.

3. Pros and cons of each proposition

Prof. Wei-Jei Lee — RYGB conversion, keep old anastomosis + circumferential suture + truncal vagotomy

Pros

- Highest declared track record for *this exact* procedure (OAGB pioneer since 2000; ~30 identical cases; recent complication rate reported near zero).
- Lowest-trauma operation: no new anastomosis to leak, no drain/NG, 1–2 day stay, fast recovery — attractive given the patient's anaemia and long travel.
- Engaged with the patient's detailed questions point-by-point; internally consistent surgical logic (keeps blood supply, 2 cm safety margin, tented suture to avoid stenosis).
- Addresses bile (RYGB diversion) *and* acid (vagotomy) — belt-and-braces on ulcer recurrence.

Cons

- Leaving a large, chronically ulcerated, previously-bled anastomosis in situ is the most debated choice — no tissue diagnosis, and "circumferential suturing" of an ulcer is a non-standard, hard-to-verify manoeuvre.
- **Refuses biopsy** of a chronic, bled ulcer in a 59-year-old — most guidelines would biopsy to exclude malignancy.
- Truncal vagotomy adds risk of gastric stasis/dumping; its necessity is contested once bile is diverted (see Fakhrul).
- Downplays H. pylori and skips structured nutritional/anaesthetic optimisation despite recent bleed, anaemia and complex drug list.
- 40 cm limb is shorter than many published anti-bile-reflux conversions.

Dr Usama Iqbal — RYGB conversion with resection of the ulcerated anastomosis + new anastomosis, all-in-one

Pros

- Resecting the diseased anastomosis removes the ulcer-bearing tissue outright — the most definitive answer to a chronic, bled ulcer, and allows histology.
- Clear, decisive single-stage plan (RYGB + HH repair + cholecystectomy) with sound reasons (post-RYGB gallstone complications are hard to treat).
- Most **process-disciplined**: defined pre-op lab panel, mandatory anaesthesia fitness teleclinic, named stapler platform, states he is the primary surgeon.
- Correctly frames the ulcer as bile/ischaemia-driven and defers H. pylori to long-term GI care rather than delaying surgery.
- Accessible (same region, visa support, structured coordination).

Cons

- Did not commit limb lengths in writing — the single most important variable for bile control was left to the consult.
- "~30 revisions, no complications" is a *mixed* revision figure, not OAGB□RYGB-specific; "no complications" across all revisions is an optimistic claim that invites scrutiny.
- Preferred to move discussion off email to teleclinic — reasonable clinically, but left several of the patient's written questions unanswered on paper.
- A new anastomosis carries a real (if small) leak risk that Lee's approach avoids.

Dr Murat Üstün — RYGB conversion (or reversal), ulcer managed intra-operatively, combined where safe

Pros

- **Most careful, safety-first assessment** on paper: the only one to flag the discrepancy in the medication records, insist the ulcer "should not be assumed bile-only," and keep reversal genuinely on the table.
- Most complete pre-operative pathway: nutrition panel, anaesthetic/cardiac review, specific planning for methylphenidate/hypersomnia, and colonoscopy-lesion follow-up.
- Explicitly offers a **staged option** if bleeding/anaemia/nutrition make one-stage unsafe — the most individualised risk posture.
- Transparent, itemised package pricing and realistic ~10-day timeline including fit-to-fly.
- Appropriately cautious, hedged language — avoids over-promising.

Cons

- Least *committal* surgical plan — much is deferred to "intra-operative assessment," so the patient can't compare a concrete technique.
- No specific limb length, no firm decision on resect-vs-keep, no stated vagotomy position.
- Did **not state personal/team case volumes** for OAGB revision in writing even when directly asked — a gap given the question was explicit.
- The clinical content arrives wrapped in a commercial "luxury package / special price" frame, which can blur the line between clinical advice and sales.

Dr Fakhrul Islam — advisory opinion: RYGB with a new anastomosis, specified limbs, no vagotomy, remove gallbladder

Pros

- Clear, decisive and well-reasoned on the two pivotal points: **bile — not acid — is the driver**, so convert properly and **do not do a vagotomy** (a view well supported in current practice).
- The only opinion to commit specific, bile-protective numbers: **AL 75 cm + BP 50–60 cm** with a refashioned anastomosis.
- Firm on removing the gallbladder in the same sitting and on at least a cruroplasty for the hiatus.
- Adds genuine value as a trusted, candid overview that reconciles the conflicting opinions — and honestly flags the patient's information-overload/anxiety as a decision risk.

Cons

- It is an **advisory second opinion, not an operative offer** — he would refer the patient on (suggests the USA), so there is no accountable surgeon, cost, or facility attached.
- No case-volume, no pre-op/anaesthetic work-up detail, no complication data — outside the scope of an advisory note.
- Asks the patient to "avoid highlighting all the risks" and to trust the plan — reassuring in intent, but under-weights informed consent for a high-complexity revision.
- Recommendations can't be executed as-is; they must be handed to an operating team, reintroducing the very variability he describes.

4. Scoring

Each proposition is scored out of 10 against six weighted criteria. Scores reflect the **proposal as written in the correspondence** and its alignment with mainstream OAGB-revision practice (RYGB conversion for bile reflux, resection + biopsy of a chronic bled ulcer, bile-protective limb lengths, structured peri-operative optimisation). They do *not* measure surgical skill, which cannot be judged from emails.

Criterion (weight)	Lee	Usama	Murat	Fakhrul
Soundness vs. evidence — tackles the *bile* driver, handles the ulcer correctly (25%)	6	9	8	**9**
Completeness of the plan — limb lengths, all 3 problems, concrete technique (20%)	8	7	7	**8**
Peri-operative safety & work-up — anaemia, anaesthesia, nutrition, H. pylori (20%)	5	8	9	**5**
Demonstrated relevant experience (stated) (15%)	9	7	5	**3**
Communication, transparency & informed consent (10%)	8	7	9	**6**
Deliverability — can it actually be carried out for this patient? (10%)	9	9	8	**4**

Weighted scores out of 10

Proposition	Score /10	One-line verdict
Dr Usama Iqbal	**7.9**	Most **definitive and executable** plan — resects the ulcer, disciplined process; just needs limb lengths pinned down.
Dr Murat Üstün	**7.5**	Safest, most thorough **assessment**; loses points for a deliberately non-committal surgical plan and undisclosed volumes.
Dr Fakhrul Islam	**6.6**	Sharpest **reasoning** (bile-driven, no vagotomy, specified limbs) — but advisory only, not an operative offer.
Prof. Wei-Jei Lee	**6.9**	Strongest **track record** and lowest-trauma operation; held back by keeping an unbiopsied ulcer and a contested vagotomy.

Score visualised



How to read these scores. They are close on purpose — all four converge on the right core operation (RYGB conversion). The gaps come from the *details*: Usama and Fakhrul make the technically strongest ulcer/bile choices; Murat gives the safest work-up; Lee offers the strongest experience and the gentlest operation but the most debatable ulcer decision. A reasonable reading is that the **best plan may combine elements**: Usama's/Fakhrul's resection + bile-protective limb lengths, delivered with Murat's peri-operative rigour — a point worth raising directly with whichever surgeon the patient chooses.

5. Suggested questions to settle before deciding

- **Resect or keep the ulcer?** Ask each surgeon to justify, and ask specifically about biopsy to exclude malignancy in a chronic, bled ulcer.
- **Exact limb lengths** (alimentary + biliopancreatic + common channel) and the evidence for the bile-reflux target.
- **Vagotomy — yes or no?** If yes, what is the plan for post-vagotomy diarrhoea/stasis, and why is it needed once bile is diverted?
- **Procedure-specific outcomes**: leak, bleed, re-operation and mortality rates for OAGB□RYGB specifically (not mixed revisions).
- **Peri-operative optimisation**: anaemia correction, anaesthetic plan around methylphenidate/modafinil, H. pylori confirmation, and whether the ulcer must heal first.
- **One-stage vs staged** given the recent bleed — and the fall-back if intra-operative findings change the plan.

Sources: the four Word documents in the repository MMS-ITS/Revision_Surgery — *Emails Prof. Wei-Jei Lee*, *Emails with Dr. Usama Iqbal (Aga Khan University Hospital)*, *Emails Istanbul Bariatric Center Team*, and *Opinion of Dr Fakhrul Islam*. Clinical framing reflects mainstream bariatric-revision practice as of the research date.

Generated as an educational comparison on 8 November 2026. Not medical advice. Verify every clinical point with the treating team.